CLINICAL LABORATORY & ASSESSMENT REPORT

SYNTHETIC TEST DATA - NOT A REAL MEDICAL RECORD

Report ID: DEMO-T2D-2026-001 **Report date:** 29 October 2026
Patient: Anonymous simulated case **Record type:** Educational testing sample

Clinical overview

Simulated adult case presenting with increased thirst, frequent urination, fatigue, and intermittent blurred vision over several weeks. Laboratory values below are intentionally constructed to illustrate findings consistent with type 2 diabetes mellitus. No real person was examined.

Laboratory findings

Test	Simulated result	Reference / interpretation
Fasting plasma glucose	168 mg/dL	Normal: 70-99 mg/dL
HbA1c	8.2%	Diabetes threshold: >=6.5%
Random plasma glucose	224 mg/dL	>=200 mg/dL with classic symptoms supports diagnosis
Creatinine	0.94 mg/dL	Typical adult range: ~0.6-1.3 mg/dL
eGFR	96 mL/min/1.73 m2	Generally >=90 considered normal
Total cholesterol	218 mg/dL	Desirable: <200 mg/dL
LDL cholesterol	139 mg/dL	Optimal: <100 mg/dL
HDL cholesterol	39 mg/dL	Low in many adult reference frameworks
Triglycerides	186 mg/dL	Normal: <150 mg/dL

Simulated clinical interpretation

Elevated fasting glucose and HbA1c are consistent with diabetes-range hyperglycemia. The lipid profile also shows elevated LDL cholesterol and triglycerides. Kidney filtration markers in this fictional case are within commonly used adult ranges. Diagnosis and management in real patients require clinician assessment and appropriate confirmation.

Illustrative management considerations

A clinician might assess cardiovascular risk, discuss nutrition and physical activity, review medication options, arrange retinal and foot screening, and repeat HbA1c in approximately three months. This paragraph is educational sample content, not an individualized prescription.

Testing note: All values, symptoms, identifiers, and interpretations are synthetic. This document must not be used for clinical decisions, insurance, employment, or patient care.